

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 3, Open-label, Randomized, Noninferiority Trial of Subcutaneous Formulation of Nivolumab Versus Intravenous Nivolumab in Participants With Advanced or Metastatic Clear Cell Renal Cell Carcinoma Who Have Received Prior Systemic Therapy
Short Title:	A Study of Subcutaneous Nivolumab Versus Intravenous Nivolumab in Participants With Previously Treated Clear Cell Renal Cell Carcinoma That is Advanced or Has Spread
Protocol Number:	777995428491084814
NCT Number:	NCT04810078
Sponsor Name:	Bristol Myers Squibb
Investigational Product:	nivolumab
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Time-averaged serum concentration over 28 days (Cavgd28) 2. Trough serum concentration at steady-state (Cminss)
Secondary Obj.:	1. Objective response rate (ORR) by Blinded Independent Central Review (BICR) with a minimum of 6 months follow-up 2. Trough serum concentration at day 28 (Cmind28) 3. Maximum serum concentration after the first dose (Cmax1)

2.2 Background & Rationale

The purpose of this study is to evaluate the drug levels, efficacy, safety, and tolerability of subcutaneous nivolumab versus intravenous nivolumab in participants with previously treated clear cell renal cell carcinoma that is advanced or has spread. The purpose of this study's substudy is to evaluate drug level biocomparability of subcutaneous nivolumab manufactured using two different manufacturing processes.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: ACTIVE_NOT_RECRUITING
Target N:	681

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Histological confirmation of renal cell carcinoma (RCC) with a clear cell component, including participants who may also have sarcomatoid features
- * Advanced RCC (not amenable to curative surgery or radiation therapy) or metastatic RCC (Stage IV)

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- * Measurable disease as defined by Response Evaluation Criteria in Solid Tumor (RECIST) v1.1 criteria within 28 days prior to randomization
- * Received no more than 2 prior systemic treatment regimens
- * Intolerance or progression on or after the last treatment regimen received and within 6 months prior to randomization
- * Karnofsky PS $\geq$ 70 at screening
- * Must agree to follow specific methods of contraception, if applicable

4.2 Exclusion Criteria

- * Untreated, symptomatic central nervous system (CNS) metastases
- * Concurrent malignancy (present during screening) requiring treatment or history of prior malignancy active within 2 years prior to randomization
- * Active, known, or suspected autoimmune disease
- * Known human immunodeficiency virus (HIV) positive with an acquired immunodeficiency syndrome (AIDS) defining opportunistic infection within the last year, or a current CD4 count $<$ 350 cells/ μ L. Participants with HIV are eligible if:

1. They have received established antiretroviral therapy (ART) for at least 4 weeks prior to randomization
 2. They continue on ART as clinically indicated while enrolled on study
 3. CD4 counts and viral load are monitored per standard of care by a local health care provider
 4. Inclusion of participants with HIV should be based on Investigator clinical judgment in consultation with the Medical Monitor NOTE: Testing for HIV must be performed at sites where mandated locally. HIV-positive participants must be excluded where mandated locally
- * Serious or uncontrolled medical disorders including for example, active severe acute respiratory syndrome coronavirus 2 (SAR-CoV-2) infection within approximately 4 weeks prior to screening. In the case of prior SARS-CoV-2 infection, acute symptoms must have resolved based on investigator clinical judgment and, in consultation with Medical Monitor, there are no sequelae that would place the participant at a higher risk of receiving investigational treatment to be eligible
 - * Prior treatment with an programmed death receptor-1 (anti-PD-1), programmed death ligand-1 (anti-PD-L1), or cytotoxic T-lymphocyte-associated antigen-4 (anti-CTLA-4) antibody, or any other antibody or drug specifically targeting T-cell costimulation or checkpoint pathways
 - * Treatment with any live attenuated vaccine within 30 days of first study treatment

Other protocol-defined inclusion/exclusion criteria apply

11. Study Identifiers & Governance

Primary ID:	777995428491084814
Registry Number:	NCT04810078
Sponsor:	Bristol Myers Squibb
Study Title:	A Study of Subcutaneous Nivolumab Versus Intravenous Nivolumab in Participants With Previously Treated Clear Cell Renal Cell Carcinoma That is Advanced or Has Spread

15. Sign-off & Approval

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Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____